MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 13 OF 28

MFT 713

Substance Abuse and Family Dynamics

15 teaching weeks · 4 practicals · 1 worked cases · 2 figures

SEMESTER TWO

1.71 units · Prerequisite: MFT 710

> “Addiction is a family-organising process.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 713 — Substance Abuse and Family Dynamics

MSc Marriage and Family Therapy · Semester Two · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 710

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	**Substance Abuse and Family Dynamics**	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Addiction is a family-organising process. This course teaches you to work with the user, the family, and the household system — using motivational, behavioural, systemic and recovery-oriented approaches — without moralising, colluding or abandoning hope.

How to use this book

Read the case at the start of each week before the theory. Addiction work punishes abstraction: families are exhausted, professionals are frustrated, and the evidence is often misrepresented by both. Hold two principles throughout: **the person is not the substance**, and **the family is not the cause**.

1. Course description

The course covers: models of addiction (disease, learning, biopsychosocial, systemic); assessment; the family cycle of use, secrecy and shame; motivational interviewing; relapse prevention; behavioural couples therapy for substance use; CRAFT (for concerned relatives); the interface with mental illness ("dual diagnosis"); recovery as a family process; medication-assisted treatment; and policy and cultural context.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain and compare models of addiction, including their clinical implications	Exam
LO2	Conduct a substance-use assessment, including readiness to change	Observed assessment
LO3	Use motivational interviewing skills with fidelity	Skills test
LO4	Formulate the family's role in maintaining and resolving use	Case study
LO5	Deliver CRAFT or behavioural couples work for substance use	Case study
LO6	Work with dual diagnosis and coordinate with medical and recovery services	Viva
LO7	Address stigma, culture and shame in your own practice	Reflective portfolio

3. Assessment

Component	Weight	Due
Motivational interviewing observed (25 min)	25%	Week 5
Family formulation and intervention plan (2,500 words)	35%	Week 11
Examination	25%	Week 16

Component	Weight	Due
Reflective portfolio (6 entries)	15%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	Models of addiction and their consequences	Model comparison debate
2	The family cycle: use, secrecy, enabling, crisis, shame, promise	Cycle mapping
3	Assessment: quantity, function, consequences, readiness	Structured interview
4	Motivational interviewing I: OARS and the spirit	OARS drill
5	**MI observed**; MI II: change talk and discord	Change-talk practice
6	The stages of change and the family's stage	Stage-matched planning
7	Working with the user: CBT and relapse prevention	Relapse prevention plan
8	Working with the couple: behavioural couples therapy	Daily recovery plan
9	Working with relatives: CRAFT and boundaries	CRAFT role-play
10	Co-dependency, enabling and the limits of the concept	Boundary-setting lab
11	**Case plan due**; dual diagnosis	Integrated plan
12	Medication-assisted treatment and the medical interface	Referral exercise
13	Recovery, relapse and the family's long haul	Relapse response plan
14	Culture, faith, stigma and the community	Community resource map
15	Integration: what works, for whom	Case presentations

Week 1 — Models of addiction

Model	Core claim	Clinical implication	Critique
Moral	Use is a choice; user is weak or bad	Punishment, exclusion	Shame increases use; ignores biology
Disease / medical	Addiction is a chronic brain disease	Treatment, medication, compassion	Can reduce agency; "once an addict always" fatalism
Learning / behavioural	Use is reinforced behaviour	Skills, contingency management	Can ignore neurobiology and context
Biopsychosocial	Biology × psychology × social context	Multimodal treatment	Descriptive rather than prescriptive
Systemic / family	Use is embedded in and organises relationships	Family-inclusive treatment	Risk of blaming the family

Model	Core claim	Clinical implication	Critique
Recovery / social	Recovery is a social process, not only clinical	Peer support, community, identity	Needs availability of recovery capital

The position used here. Addiction is a **chronic, relapsing condition** shaped by biology, learning, trauma, relationship, culture and economics. The person retains agency but not unlimited agency. Families are neither causes nor cures; they are the environment in which recovery either becomes possible or does not.

> **DEBATE**
> "Addiction is a disease" versus "addiction is a choice". Take a side, then argue the other side, then write the practical position you will hold with clients.

Week 2 — The family cycle

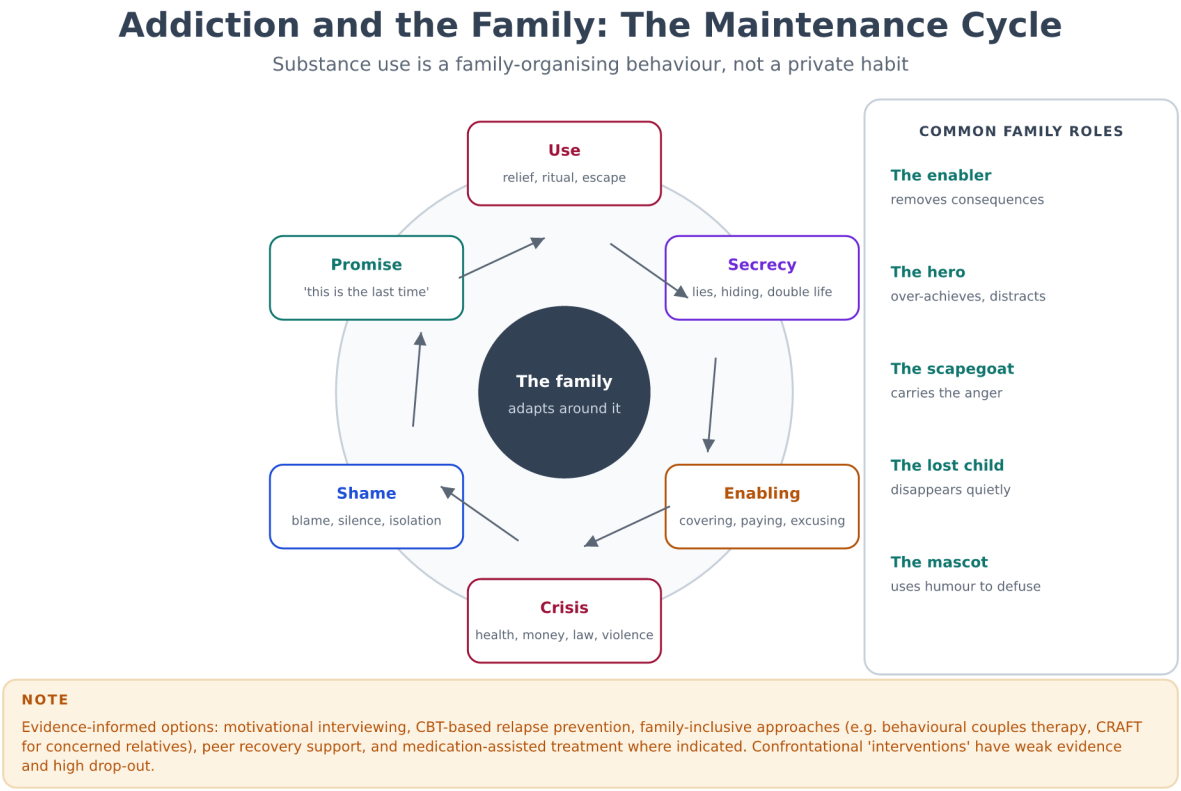


Figure 2.1 — The addiction and family maintenance cycle

Six stations that repeat: use → secrecy → enabling → crisis → shame → promise → use.

Enabling is not weakness. It is a set of behaviours that reduce immediate consequences because the alternative (the person losing their job, being arrested, or dying) feels unbearable: paying a debt, calling in sick, hiding the bottles, giving money "for food".

Family roles (an imperfect but useful map): the enabler, the hero, the scapegoat, the lost child, the mascot. Use them as questions, never as labels: "Who in the family keeps the peace? Who takes the blame? Who disappears?"

The systemic point. The cycle is maintained by both the using and the accommodating. This is not a statement of blame; it is a statement of leverage. Change the accommodating behaviour and the cycle must change.

> **PRACTICAL**
> Map a family cycle from a disguised case. At each station, write: who does what, what they feel, and what they fear. Then identify two points of leverage.

Week 3 — Assessment

Substance history.

Domain	Questions
Substances	What, how much, how often, by what route, for how long
Pattern	Time of day, alone or social, binge or daily
Function	What does it do for you? (sleep, courage, relief, numbness, belonging)
Consequences	Health, money, work, relationships, legal, children
Control attempts	Have you tried to stop? What happened? How long?
Withdrawal	Symptoms, previous seizures, previous medically assisted withdrawal
Dependence	Tolerance, withdrawal, compulsion, time spent, continued despite harm
Readiness	Do you want to change? Why now? What would it take?
Risk	Suicide, violence, driving, overdose, child safety

Validated tools. AUDIT (alcohol), DUDIT (drugs), ASSIST (multiple substances), CRAFFT (adolescents). Use them as a structure for conversation, not a verdict.

Assess the family separately. With the user absent: what has been tried, what has been covered up, what each member fears, and who is at risk.

Week 4 — Motivational interviewing I

The spirit (Miller & Rollnick): partnership, acceptance, compassion, evocation. You are not the expert on what they should do; you are the expert on a conversation that helps them say it themselves.

The four core skills (OARS).

Skill	What it is	Example
Open question	Invites elaboration	"What is a typical evening like?"
Affirmation	Names a strength, specifically	"You kept going to work through all this."
Reflection	Returns content or feeling	"You are worn out by the effort it takes to keep this hidden."
Summary	Collects and connects	"So on one hand it helps you sleep; on the other it is costing you your marriage."

What to avoid. The righting reflex (the urge to fix), confronting, warning, advising without permission, and arguing. When you argue for change, the client argues against it — and you have made change less likely.

Reflection is the active ingredient. Aim for two reflections for every question, and for complex reflections (which add meaning) over simple ones (which repeat content).

> **PRACTICAL**
> OARS drill: 8 minutes, no advice, no questions after the first one. Observer counts reflections, and rates how many were complex.

Week 5 — Motivational interviewing II

Change talk — client statements in favour of change. Four types: desire ("I want to"), ability ("I could"), reasons ("because…"), need ("I have to"). Also mobilising talk: commitment ("I will"), activation ("I am ready"), taking steps ("I bought the medication").

Discord is not resistance; it is a signal about the relationship. It appears as arguing, interrupting, discounting, ignoring. The response is not confrontation but **rolling with it**: "You are not sure this is worth it. Tell me more about that."

The MI-consistent techniques.

- **Elicit–provide–elicit:** "What do you already know about what alcohol does to the liver?" → brief, neutral information → "What does that mean for you?"
- **The decisional balance:** "What are the good things about using? And the less good things?" (Ask in that order, always.)
- **Looking forward:** "If nothing changes, where do you imagine you will be in five years?"
- **Readiness ruler:** "On 0–10, how important is it to change? How confident are you that you could? What would move you from 4 to 6?"

Avoid the readiness trap: a score of 3 is an argument for a smaller intervention, not for discharge.

ASSESSED

25-minute MI session with a trained role-player. Rated on: spirit, OARS fidelity, ratio of reflections to questions, and handling of discord.

Week 6 — Stages of change and the family

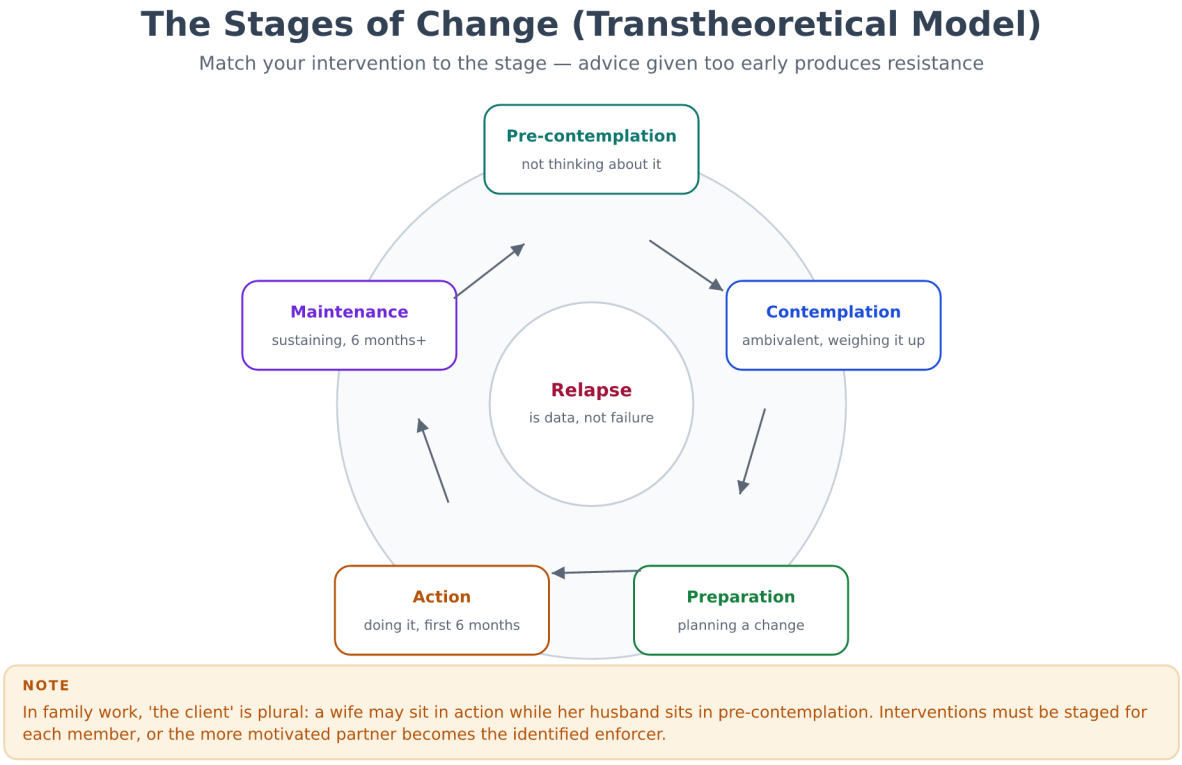


Figure 6.1 — The stages of change, including relapse

Stage	The person	The family member	Your task
Pre-contemplation	Not thinking about change	Frustrated, angry, exhausted	Raise doubt gently; do not push
Contemplation	Ambivalent	Hopeful and suspicious	Tip the balance; build confidence
Preparation	Planning	Mobilising	Concrete plan; remove barriers
Action	Changing	Adjusting, testing trust	Support, skills, relapse prevention
Maintenance	Sustaining	Rebuilding	Protect gains; plan for risk
Relapse	Using again	Devastated, "I told you"	Reframe as data; re-plan fast

The family's stage often differs from the user's. A wife in action married to a husband in pre-contemplation becomes the "enforcer", which damages the relationship and does not work. Stage-matched intervention means working separately where necessary.

Week 7 — Relapse prevention

The model (Marlatt & Gordon). Relapse is a process, not an event: high-risk situations → poor coping → lapse → the abstinence violation effect ("I've blown it, so I may as well continue") → relapse.

The plan.

1. **High-risk situations** — people, places, times, feelings (loneliness, anger, celebration, payday).
2. **Coping strategies** for each.
3. **A lapse protocol** — what to do in the first 24 hours, and whom to call.
4. **Lifestyle balance** — the "shoulds" versus the "wants"; boredom and resentment are relapse risks.
5. **The family's role** — agreed, non-punitive responses to a lapse, decided in advance.

With the family. A relapse response agreed in advance prevents the cycle of crisis, shame and secrecy from restarting. Write it down: "If I use again, we will…"

Week 8 — Behavioural couples therapy for substance use

The evidence. Behavioural couples therapy (BCT) for alcohol and drug problems has good evidence, particularly where the partner is non-using and the relationship is not violent.

The "daily recovery plan" (O'Farrell). Each day: a shared trust discussion in which the abstinent partner states their intention and the other expresses support, then a specified rewarding activity together. It is behavioural, daily, and builds the relationship as a recovery resource.

Why couples work helps. Partners are often the strongest motivator and the most reliable monitoring system — but only if the relationship becomes a source of reward rather than a site of surveillance.

Contraindications. Current violence, coercive control, a partner with active untreated addiction, or a relationship the client is being coerced to remain in.

Week 9 — Working with relatives: CRAFT

CRAFT (Community Reinforcement and Family Training) works with the concerned significant other to change the contingencies around the user's behaviour, without confrontation. It has better evidence for treatment entry than traditional "interventions" or Al-Anon-style support alone.

Three goals. 1) Improve the relative's own wellbeing; 2) reduce the user's substance use; 3) increase the chance the user enters treatment.

Core skills taught to the relative.

- **Functional analysis:** what triggers use, what consequences follow, what the relative does.
- **Positive communication:** brief, specific, warm.
- **Allow natural consequences:** stop removing consequences where it is safe.
- **Reward non-use:** notice and reinforce sober days.
- **Domestic safety:** never ask a relative to accept violence.
- **Self-care and social support:** the relative's life must not be entirely about the user.
- **Invitation to treatment:** a script, rehearsed.

> **PRACTICAL**
> CRAFT role-play: a mother of a 24-year-old cannabis user. Teach positive communication and one natural consequence. Assessor rates warmth, specificity and safety.

Week 10 — Enabling, co-dependency and boundaries

"Co-dependency" is a contested term. Used carefully it describes patterns of over-functioning and self-neglect in relationships with an addicted person. Used carelessly it blames the relative — usually a woman — for the addiction.

Better language: accommodation and boundary. Ask what the relative does, what it costs them, and what they would like to do differently.

Boundary setting that works. Specific, behavioural, enforceable, and about the relative's own actions: "I will not lend money. If you come home intoxicated, I will sleep in the other room and we will talk in the morning." Not: "You must stop drinking or I will leave." (An unenforceable ultimatum teaches the family that threats are empty.)

Week 11 — Dual diagnosis

The overlap is the rule. Anxiety, depression, PTSD, ADHD and psychosis commonly co-occur with substance use. Direction of causality varies; the treatment must be integrated rather than sequential.

Principles.

1. Treat both, at the same time, by the same team where possible.
2. Do not require abstinence before treating mental illness, and do not require remission of mental illness before treating addiction.
3. Be cautious with medication (benzodiazepines, stimulants) and coordinate with the prescriber.
4. Distinguish substance-induced symptoms from independent disorders (timing and persistence).
5. Trauma is common; assume it until assessed.

CASE PLAN (35%)
A family case with dual diagnosis: assessment, formulation, integrated plan, coordination with medical and recovery services, risk plan, and outcome measures.

Week 12 — Medication-assisted treatment

Opioid agonist therapy (methadone, buprenorphine) reduces mortality and improves retention; it is evidence-based treatment, not "substituting one drug for another". **Acamprosate** and **naltrexone** reduce alcohol relapse; **disulfiram** works only with supervision. Withdrawal management alone is not treatment.

The family therapist's role. Support adherence, address stigma within the family, challenge the belief that medication is cheating, and coordinate with the prescriber. Do not advise on doses or discontinuation.

Week 13 — Recovery as a family process

Recovery capital (Cloud & Granfield): social (relationships, support), physical (health, housing), human (skills, education, health literacy), cultural (values, belonging). Treatment that builds recovery capital predicts better outcomes than treatment that focuses only on symptoms.

The family's long haul. Early recovery is often called "the pink cloud"; the harder work is at six to eighteen months, when the family must renegotiate roles that were organised around the addiction. A partner who has run everything may not want to hand back control; a child who has been the carer may not want to be a child. Name this, and plan for it.

Relapse is data. The question is not "why did you fail?" but "what happened, and what does the plan need?"

Week 14 — Culture, faith, stigma and community

- **Substance use in context:** alcohol is culturally embedded in many Nigerian settings (burial rites, festivals, business); kola, cannabis, tramadol and codeine have specific local patterns.
- **Faith:** deliverance and prayer can be powerful resources and can also delay medical care. Coordinate, do not compete.
- **Stigma:** "a disgrace to the family" keeps use hidden and prevents early help. Your language matters: "person with a substance use disorder", not "addict".
- **Community resources:** mutual aid (AA/NA and their local equivalents), faith-based recovery groups, community health workers — map what actually exists near your clinic.

Week 15 — Integration

What works: screening and brief intervention; MI; CBT-based relapse prevention; BCT and CRAFT; medication where indicated; integrated treatment for dual diagnosis; recovery support; and continuity over time. What does not: confrontation, "tough love" as a single strategy, sequential treatment for dual diagnosis, and asking a family to do the work alone.

Glossary

Abstinence violation effect — the spiral of guilt and continued use after a lapse. **CRAFT** — Community Reinforcement and Family Training, for concerned significant others. **Change talk** — client statements favouring change. **Dual diagnosis** — co-occurring substance use and mental disorder. **Enabling** — behaviours that remove the natural consequences of use. **Harm reduction** — interventions that reduce harm without requiring abstinence. **Motivational interviewing** — a collaborative, directive method for strengthening intrinsic motivation. **Recovery capital** — the resources a person can draw on to sustain recovery. **Relapse** — return to problematic use after a period of change.

Self-assessment

Multiple choice

1. The righting reflex refers to: (a) the client's resistance (b) the clinician's urge to fix (c) a CBT technique (d) a legal duty

2. CRAFT is aimed primarily at: (a) the user (b) concerned relatives (c) employers (d) courts

3. Change talk includes: (a) sustain talk (b) desire, ability, reasons, need (c) only commitment (d) only questions

4. Medication-assisted treatment is: (a) substituting one drug for another (b) evidence-based treatment that reduces mortality (c) only for the wealthy (d) discouraged

5. The "abstinence violation effect" describes: (a) legal consequences (b) the spiral after a lapse (c) withdrawal (d) family conflict

6. A well-formed boundary is: (a) an unenforceable ultimatum (b) specific, behavioural and about the relative's own actions (c) a threat (d) a diagnosis

7. In dual diagnosis, treatment should be: (a) sequential (b) integrated (c) only psychiatric (d) only addiction-focused

8. Enabling is best understood as: (a) malice (b) reducing immediate consequences out of care (c) a diagnosis (d) irrelevant

9. Relapse should be treated as: (a) failure (b) data for re-planning (c) grounds for discharge (d) proof of character

10. Recovery capital includes: (a) only abstinence (b) social, physical, human and cultural resources (c) only income (d) only family support

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Compare the disease and systemic models of addiction and name one clinical implication of each.
2. Map the six stations of the family cycle and give one intervention at each.
3. What are OARS, and what is the recommended ratio of reflections to questions?
4. Describe the daily recovery plan in behavioural couples therapy.
5. Name the three goals of CRAFT.

Essay (choose one, 2,000 words)

1. "The family is neither the cause nor the cure, but it is the context." Discuss with a case.
2. Critically evaluate the concept of co-dependency and its gendered assumptions.
3. Design a family-inclusive service for adolescents with substance use problems in a Nigerian city.

Further reading

- Miller, W. R., & Rollnick, S. (2013). Motivational Interviewing (3rd ed.). Guilford.
- O'Farrell, T. J., & Fals-Stewart, W. (2006). Behavioral Couples Therapy for Alcoholism and Drug Abuse. Guilford.
- Smith, J. E., & Meyers, R. J. (2004). Motivating Substance Abusers to Enter Treatment: Working with Family Members. Guilford.
- Marlatt, G. A., & Donovan, D. M. (2005). Relapse Prevention (2nd ed.). Guilford.
- Cloud, W., & Granfield, R. (2008). Conceptualizing recovery capital. Substance Use & Misuse, 43(12–13).

Next: MFT 714 — Trauma-Informed Care